



Clinical Practice Guideline

Cognitive Impairment Management (Delirium and/or Dementia)

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisational wide	All Staff

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Introduction

The Sir Charles Gairdner Osborne Park Health Care Group (SCGOPHCG) acts on evidence that supports early identification and provision of individualised care for patients with cognitive impairment (dementia and/or delirium).

Cognitive screening is the most important strategy to improve safety and to reduce risk of harm for this cohort of patients.

Early identification, reporting and assessment of delirium is vital as it is well recognised that Delirium can be a trigger/flag for a medical emergency.^{1, 2}

This Clinical Practice Guideline establishes the minimum standards for the screening, assessment and management of patients with a cognitive impairment at SCGOPHCG and is aligned with the [Australian Commission on Safety and Quality in Health Care Delirium Clinical Care Standards](#).

Risk Statement

Non-compliance with this Clinical Practice Guideline will:

Breach legislative requirements	☐	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☒	Misconduct	☐
Breach SCGH Mission & Values	☒	Staff Safety	☐
Other:	☐		

Definitions

Cognitive Impairment	An overarching term referring to deficits in one or more of the areas of memory, problems with communication, attention, thinking and judgement. Dementia and delirium are common forms of cognitive impairment seen in older hospitalised patients. ¹ Other causes include stroke, acquired brain injury, depression and intellectual disability.
Dementia	A progressive decline in cognitive function that affects memory, judgement, attention, language and problem solving. It is usually gradual, progressive and irreversible, leading to impaired functioning. ¹
Delirium	An acute change in mental status characterised by a disturbance in consciousness, attention, cognition and perception that develops over a short period of time usually over hours or days. Delirium is a serious condition and should be treated promptly and appropriately. ²

General Principles

Identification of cognitive impairment should follow the Cognitive Impairment Pathway - See [Appendix 1](#).

1. Initial Screening

Each patient will be screened for cognitive impairment within 8 hours of admission using the 4 As test (4AT) as part of their admission assessment.

Pre-admission cognitive screening conducted over the telephone will use the Ottawa 3 Day Year (O3DY) tool.

Other cognitive screening tools commonly used at SCGOPHCG include:

- Abbreviated Mental Test (AMT)
- Confusion Assessment Method – Intensive Care Unit (CAM-ICU) with Richmond Agitation Sedation Scale (RASS)
- Montreal Cognitive Assessment (MoCA)
- Ottawa 3DY

See [Appendix 2](#) for Screening Tool details.

Patients identified with a Cognitive Impairment will be recognised via a visual alert in the patient's room and on the journey/white board:

See [Appendix 5](#) for Cognitive Impairment Identifier.

2. Re-screening

It is the responsibility of each professional group to complete timely cognitive screening.

Screening must be continued throughout a patient's admission, particularly if:

- The patient has a sudden decline in cognitive functioning or a change in behaviour
- The patient moves wards
- Change in medical condition or mobility
- Post procedure/operation
- Post fall

- A pressure injury develops
- The family/carer raises concern

Patients within the intensive care unit (ICU) will be screened hourly utilising the Richmond Agitation Sedation Scale (RASS). This is documented on the ICU flow chart.

Patients with a RASS score of greater than 3 will be screened BD utilising the Confusion Assessment Method (CAM-ICU) to assess for delirium. Assessment results are documented on the ICU/HDU Flow Chart.

3. Assessment

Patients identified with cognitive impairment on presentation to hospital will be promptly assessed by a trained clinician including but not exclusive to; medical, nursing and occupational therapists, using a validated assessment tool.

See [Appendix 3](#) for Assessment Tool details.

An assessment will seek input from the patient and/or their family/carer, GP and other primary care providers about cognitive and behavioural changes such as:

- Confusion
- Decreased concentration
- Disorientation
- Agitation
- Restlessness
- Sleepiness (including altered levels of consciousness)
- Speech or language disturbance
- Cooperation with requests
- Alterations in mood
- Hallucinations

4. Prevention and Intervention

Patients with cognitive impairment will be provided a set of interventions to prevent the onset of delirium, complications of delirium, or deterioration in cognitive function.

These interventions could include:

- Nutrition and Hydration
 - Assess, encourage/assist and monitor nutrition and hydration status
 - Document fluid intake and output
 - Ensure good mouth care
 - Ensure dentures are present and well-fitting
- Bowel and bladder management⁹
 - Assess and record continence and bowel habits
 - Record post- void bladder volumes
 - Complete a urinalysis
 - Avoid use of indwelling catheters
- Mobility activities
 - Ensure Falls Risk Screen is current
 - Ensure Falls Prevention Interventions are in place
 - Encourage and assist with regular mobilisation (at least once per shift)
 - Encourage to sit out of bed
- Skin Integrity
 - Ensure Pressure Injury Risk Screen is current
 - Ensure Pressure Injury Prevention Interventions are in place
- Oxygen therapy
 - Provision of supplementary oxygen, if appropriate

- Pain assessment and management
 - Assess pain using Pain Score on Observation and Response Chart or observational/behavioural assessment for patients with severe cognitive impairment⁸
 - Use pain assessment tools that are valid and reliable for patients with cognitive impairment
 - Consider analgesic requirements and continue to assess and document in patient notes if any pain⁹
 - Use non-pharmacological methods (e.g., positioning, heat packs,)⁹
 - If opiates are used to treat pain, it is important that laxatives are prescribed to prevent constipation which can contribute to delirium⁹
- Regular reorientation and reassurance
 - Remind of the location and time of day
 - Respond to feelings of anxiety and/or fear
 - Ensure that hearing and visual aids fit and are in good working order.
 - Provide pen and paper for hearing or speech impairment
- Activities for stimulating cognition
- Improve Sleep Hygiene ^{3,4}
 - Provide a quiet and relaxing environment
 - Minimise caffeinated drinks in the afternoon
 - Provide natural light where possible
 - Limit daytime naps
 - Use night lights to assist with orientation/mobilisation
- Environmental modification and support
 - Ensure appropriate lighting for the time of day
 - Keep a calendar and/or clock within view
 - Establish daily routines for Activities of Daily Living
 - Implement a plan of management for violence and aggression.
 - Minimise room and ward changes- transfer to occur between 0700 and 1500 hrs^{4,10}
- Key communication strategies ^{1-3,5}
 - Introduce self
 - Make sure you have eye contact at all times
 - Remain calm and speak clearly
 - Keep sentences short and simple
 - Focus on one instruction at a time
 - Give time for responses
 - Check for understanding of instructions
 - Do not give too many choices
 - Involve family/carers
 - Regular visiting
 - Bringing in personal and familiar objects
 - Providing topics of interest e.g. Via Sunflower tool ¹²
 - Demonstrate an attitude of calmness with a soothing voice, soft touch and comforting behaviour

These interventions should be discussed with the patient and their family/carer and documented in the patient's medical record.

Patient centred care plans should be provided along with general educational materials.

Identifying and treating underlying causes

Patients identified with delirium should undergo medical review to systematically identify and treat underlying causes e.g. infection, hypoxia, medication-related, dehydration,

electrolyte imbalance, constipation etc. This may include a referral to Geriatrics and/or Older Adult Mental Health Liaison Service.

Medical staff are encouraged to utilise MR887 Behaviours of Concern (BoC) Escalation Pathway which provides guidance for assessment and management (see [Appendix 6](#)).

Patients should be monitored regularly for changes in their cognition and behaviours to identify and address clinical deterioration.

If the patient is agitated offer emotional support and other non-pharmacological strategies, involving carers if possible, to calm the patient and de-escalate the situation. Additional support staff e.g. Assistants in Nursing or Patient Care Assistants should be used as indicated.

The use of physical restraints should be avoided as they can increase agitation and increase the risk of injury. Should they be necessary, as a last resort, they should be used in accordance with the Mechanical Restraint of Patients Policy.

5. Minimising the use of antipsychotic medicines

Antipsychotic medicines have a number of serious adverse effects for older people and can worsen cognitive impairment.¹¹

Antipsychotic medicines should only be prescribed for patients whose behavioural or emotional state poses a threat to themselves or others despite management using non-pharmacological strategies¹¹

Benzodiazepines usually make agitation worse in delirium and increase the risk of falls, aspiration etc.

If non-pharmacological strategies are ineffective and an antipsychotic medicine is being considered:

- Refer to pharmaceutical guidelines and WA [Statewide Medicines Formulary](#) before prescribing
- Discuss the choice of antipsychotic medicine, the risks and benefits, dosage, and duration with the patient and carer
- A comprehensive medication review should be undertaken with input from the ward pharmacist
- Commence with a low dose and closely monitor the effect before considering any dose increases
- Limit use for as short a period as possible
- Baseline and weekly ECG to monitor QT interval if continued antipsychotic use is required or combined with medications which may prolong QT eg escitalopram
- Ensure a de-prescribing regime is considered

See [Appendix 5](#) for the Management of Agitation in Older Patients on Medical and Surgical Wards.

6. Discharge

Planning for discharge should be in collaboration with the patient and their family/carer and ideally commence at admission.

Allied Health involvement will be required if the patient is not at a functional or cognitive level required for discharge.

Ensure anti-psychotic de-prescribing plan is present in discharge letter.¹¹

Prior to discharge, the patient and their family/carer should receive:

- Referral to community services including contact details
- Individualised care plan with strategies to manage diagnosed cognitive impairment or how to prevent recurrence of delirium

- General cognitive impairment educational information
- Detailed medication information (name, dose, reason and duration)
- Discharge summary to GP listing:
 - Any concerns regarding cognition
 - ongoing treatments
 - follow-up needed
 - medicines, including details on those that were ceased or changed and a recommended plan for ongoing review/withdrawal
- Consider referral to:
 - Memory Service via Geriatric, Acute and Rehabilitation Medicine (GARM) or note on GP discharge summary
 - Community Nursing Services
 - Early Supported Discharge-Delirium
 - Occupational Therapy home visiting
 - Rehabilitation in the Home

Compliance, monitoring and evaluation

The SCGOPHCG Cognitive Impairment Working Group is responsible for monitoring compliance with this document.

Compliance is monitored via weekly Consumer Engaged National Standard Audits (CENSA) and bi-annual medical record audits.

Results are tabled at the Cognitive Impairment Working Group bi-annually and quality improvement plans tabled if warranted. Where required this will be escalated to the Comprehensive Care Committee.

Other individual audits completed if the need is identified by the Cognitive Impairment Working Group.

Documents

[Australian Commission on Safety and Quality in Health Care Delirium Clinical Care Standard 2021](#)

[SCGOPHCG Restraint Related of Patients Policy](#)

[SCGOPHCG Mechanical Restraint Minimisation CPG](#)

[SCGOPHCG High Risk Medicines Policy](#)

[SCGOPHCG CPG Pressure Injury Risk management](#)

[WA Health Nutrition Standards for Adult Inpatients in WA hospitals](#)

[WA State wide Medicines Formulary](#)

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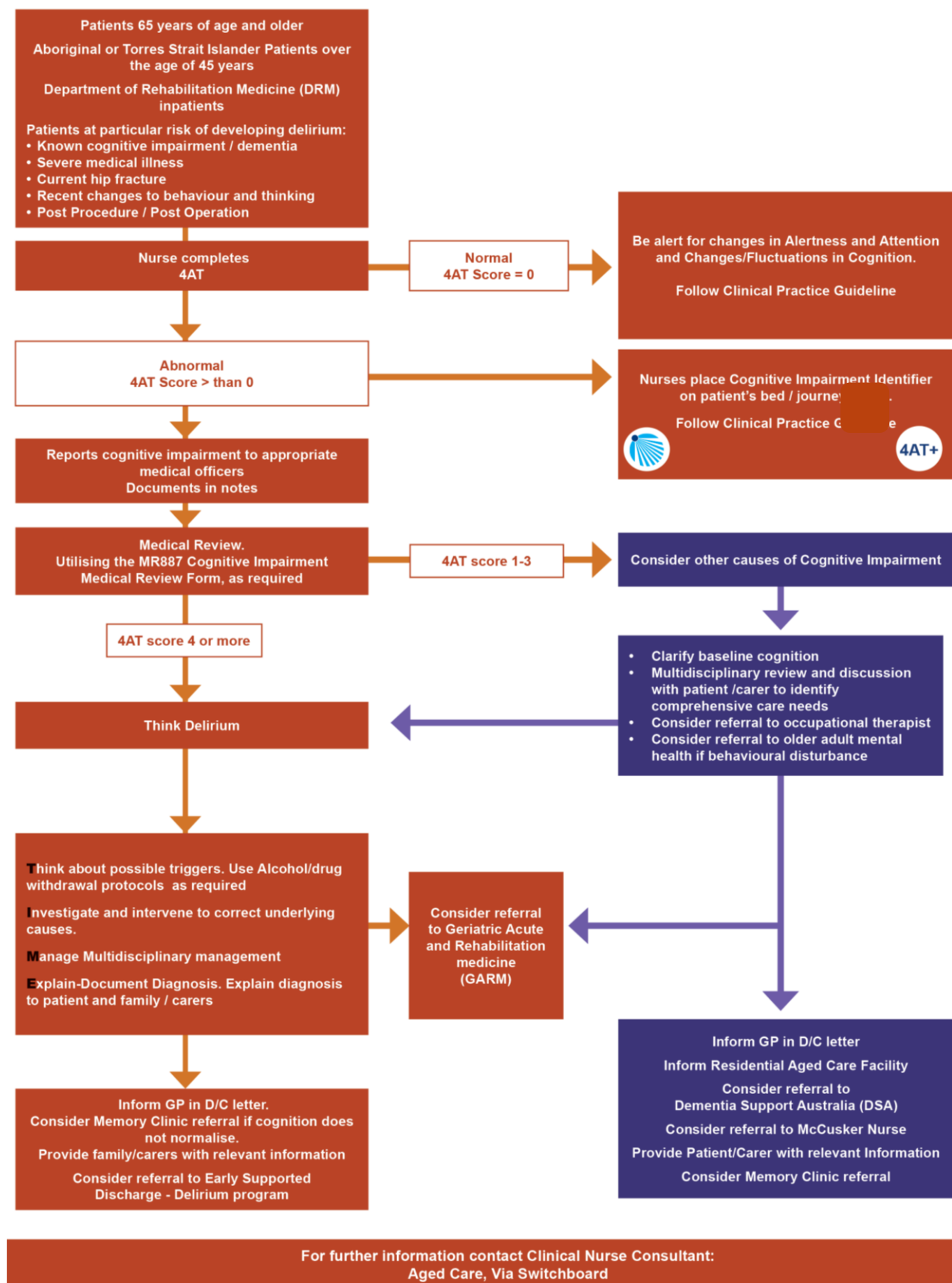
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November 2025	3.1	Comp Care Lead	Removal of Appendix 6: Medical Review Form		
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Appendix 1: Cognitive Impairment Pathway



Appendix 2: Screening tools

4AT:

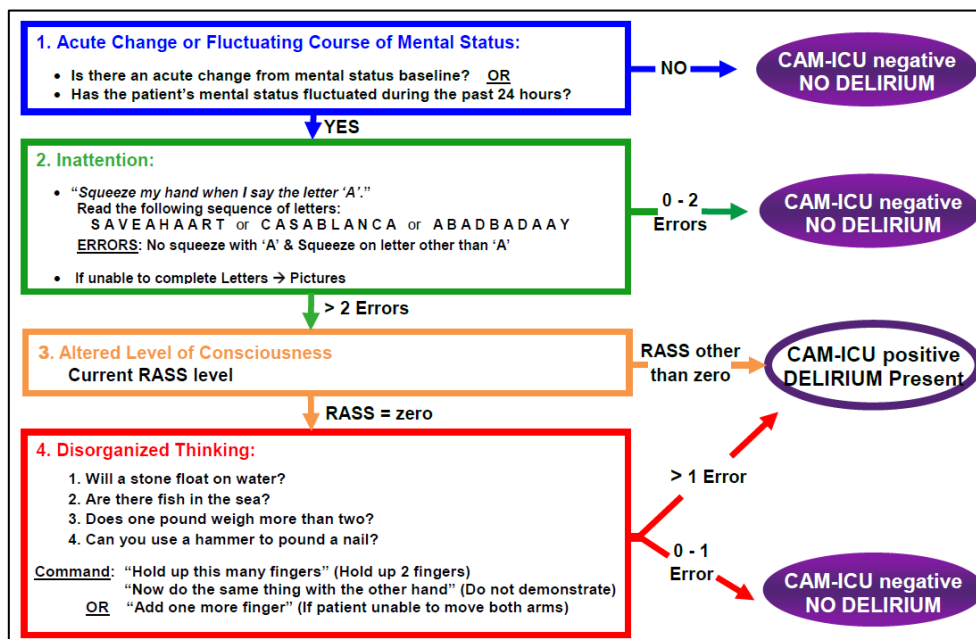
- The 4 A's test (4AT) screens **Alertness**, **AMT4**: Abbreviated Mental Test- 4, **Attention** and whether there is an **Acute** change. It is a rapid clinical test for delirium detection which includes brief cognitive tests and allows assessment of 'untestable' patients with severe drowsiness or agitation.
- The 4AT is the screening tool used as part of the Nursing Admission Assessment Form and Behaviours of Concern (BoC) Escalation Pathway.

COGNITION RISK SCREEN									
Date & Time									
Reason for assessment (Use reassessment codes)									
4AT Rapid Clinical Test for Delirium & Cognitive Impairment	1. Alertness	This includes patients who may be markedly drowsy (eg. difficult to rouse and/or obviously sleepy during assessment) or agitated / hyperactive. Observe the patient. If asleep, attempt to wake with speech or gentle touch on shoulder. Ask the patient to state their name and address to assist rating.							
		Normal (fully alert, but not agitated, throughout assessment)	0						
		Mild sleepiness for <10 seconds after waking, then normal	0						
		Clearly abnormal	4						
	2. AMT4	Ask the patient: Age, Date of birth, Place and Current year							
		No mistakes	0						
		1 mistake	1						
		2 or more mistakes / untestable	2						
	3. Attention	Ask the patient: "Please tell me the months of the year in backwards order, starting at December." To assist initial understanding one prompt of "what is the month before December?" is permitted.							
		Achieves 7 months or more correctly	0						
		Starts but scores <7 months / refuses to start	1						
		Untestable (cannot start because unwell, drowsy, inattentive)	2						
	4. Acute or fluctuating course	Evidence of significant change or fluctuation in: alertness, cognition, other mental function (eg. paranoia, hallucinations) arising over the last 2 weeks and still evident in last 24hrs							
		No	0						
		Yes	4						
4AT TOTAL									
Initials									
4 or above: Possible delirium +/- cognitive impairment 1-3: possible cognitive impairment 0: delirium or severe cognitive impairment unlikely (but delirium still possible if [4] information incomplete) If possible Delirium Identified inform the medical team.									

Confusion Assessment Method for the ICU (CAM-ICU) Flowsheet:

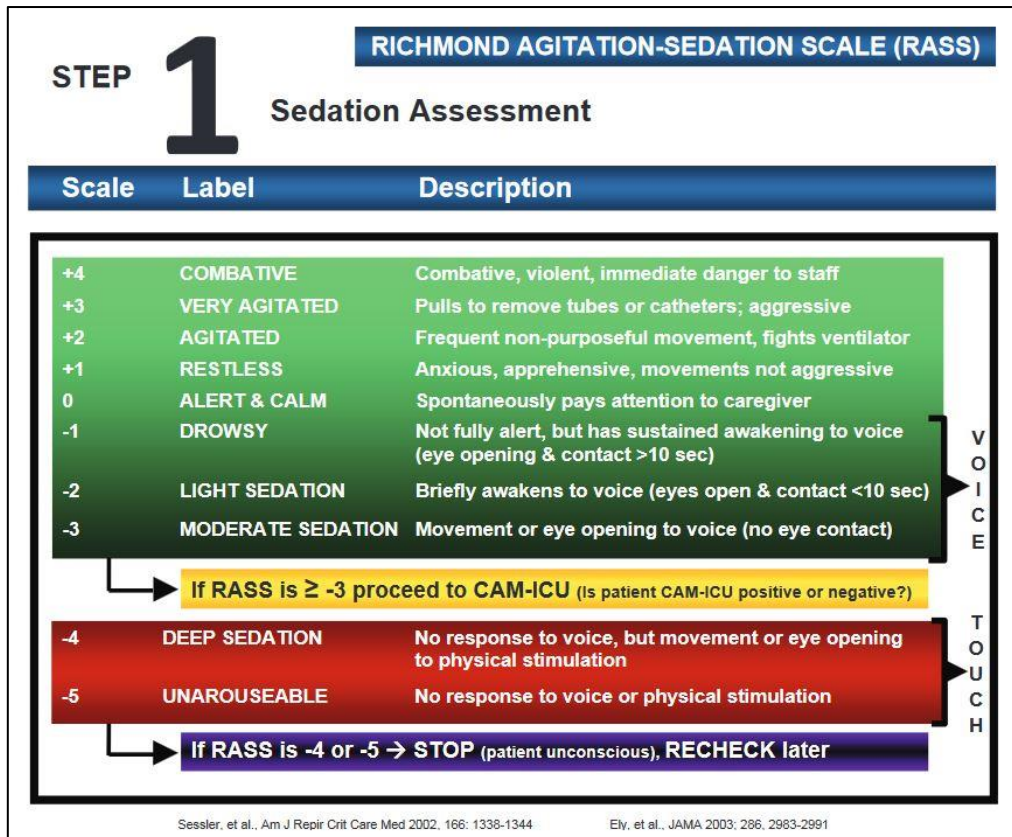
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- CAM-ICU assesses alteration/fluctuation in mental state, inattention, altered levels of consciousness and disorganised thinking and is used to help monitor patients for the development or resolution of delirium.



Richmond Agitation-Sedation Scale (RASS):

- The RASS is used for routine neurological assessments in the ICU, especially in patients without traumatic brain injury.
- The RASS is a 10 point scale with discrete criteria, with four levels of agitation (+1 to +4), one level for calm and alert state (0), and 5 levels of sedation (-1 to -5).

**Montreal Cognitive Assessment (MoCA):**

- The MoCA assesses
 - Short term memory
 - Visuospatial abilities
 - Executive functions
 - Attention, concentration and working memory
 - Language
 - Orientation to time and place
- The MoCA requires training and certification.
- A score less than 26/30 indicates possible Cognitive Impairment.
- It is available in other versions (e.g. blind) and alternative languages.

Ottawa 3 DY (O3DY) tool:

- The O3DY is a short screening tool designed to quickly identify subsets of patients at higher risk for cognitive dysfunction.
- It is composed of four questions that do not require equipment, paper, or pencil; **D**ay of the week, **D**ate, **DL**ROW (WORLD spelled backwards), and **Y**ear.

Appendix 3: Other Cognitive Impairment Screening and Assessment Tools

Tool	Description
Abbreviated Mental Test (AMT)	- The AMT rapidly assesses for mental confusion. - A score of 7–8 or less suggests CI at the time of testing although further and more formal tests are necessary to confirm a diagnosis of dementia, delirium or other causes of Cognitive Impairment. - Culturally-specific questions may vary based on region.
Abbreviated Mental Test 4 (AMT4)	- The AMT4 is a practical screening tool for cognitive impairment for older patients within a busy clinical environment. - It provides a brief assessment of cognitive function: - How old are you? - What is your date of birth? - What year is it? - Where are we? - Each correctly answered question scores 1 point. A score of less than 4/4 indicates possible Cognitive Impairment. - The AMT4 is used as part of the ED Triage Form
Clock Drawing Test (CDT)	- The CDT provides a quick assessment of visuospatial and praxis abilities and can detect deficits in both attention and executive function. Instructions (Read to the patient if necessary) - In the circle below, please draw the face of a clock and put all the numbers in the correct positions. - Then draw the hands at twenty minutes past eight.
Geriatric Depression Scale (GDS)	- The GDS is used to identify depression in older people in all settings. - Scores greater than 5 suggest the presence of depression and warrant follow up. - Scores greater than 10 are almost always depression.
Kimberley Indigenous Cognitive Assessment (KICA)	- The KICA is recommended for use with rural and remote Indigenous Australians aged 45 years and above for whom other dementia assessments are not suitable - A score of 33/39 and below indicates possible dementia. - A person scoring 33 or less should be referred to a doctor for dementia medical screens to rule out other causes of cognitive impairment.
Mini Mental State Examination (MMSE)	- The MMSE tests personal information, orientation, short-term memory, attention, naming, visuospatial and language domains. - A score of less than 24/30 suggests cognitive impairment.
Rowland Universal Dementia Assessment (RUDAS)	- The RUDAS is designed to enable the translation of the assessment into other languages and to be culturally fair. - Any score of 22 or less should be considered as possible Cognitive Impairment

Nursing Delirium Screening Scale (Nu-DESC)	• 5 areas assessed: disorientation, inappropriate behaviour, inappropriate communication, illusions or hallucinations, and psychomotor retardation • Each feature is scored on 0-2 based on severity, with 0=absent, 1=mild, and 2=severe. Positive Nu-DESC is score ≥ 2, maximum total score is 10.
Ottawa 3DY-Day/DLROW/Date/Year	• What is the Day of the week? • Spell WORLD backwards • What is the Date? • What is the Year?

Appendix 4: Management of Agitation in Older Patients with Dementia or Delirium on Medical and Surgical Wards

This guide refers to patients with persistent behavioural problems such as

- Restlessness, fidgeting, jumpiness
- Pacing
- Pulling at lines and tubes
- Trying to leave ward or hospital
- Continually trying to get out of bed
- Shouting, swearing, arguing, insulting staff
- Talking to self
- Threatening or trying to hit, kick, bite or scratch

Behaviour like this is usually due to delirium or dementia. Remember...

- Delirium may indicate a serious underlying medical problem or medication side effect.
- Refer to Cognitive Impairment (CI) Medical Review document (MR887).
- Clinical Practice Guidelines: Cognitive Impairment Management on Share Point policy page.
- Agitated patients with delirium or dementia sometimes injure themselves or staff.
- Benzodiazepines usually make agitation worse in delirium and increase the risk of falls, aspiration etc.

This guide is not about managing patients with...

- Simple insomnia
- Simple threats to leave hospital against medical advice or conflict about a specific issue
- Behavioural problems due to a psychiatric disorder such as mania or psychosis

In these situations provide counselling or discuss with a senior colleague.

Step 1. Review diagnosis and screen for new problems such as

Dehydration
Electrolyte imbalance
Level of pain
Infection/sepsis/inflammation
Respiratory failure (hypoxia/hypercapnia)
Impaction of faeces
Urine retention
Medication toxicity: especially anticholinergics (e.g. tricyclics, oxybutynin, tiotropium), digoxin, sedatives/opioids, corticosteroids, anticonvulsants, antiparkinsons)

1

If withdrawal is suspected

- Use diazepam and thiamine
- Do not use antipsychotics
- Commence Alcohol Withdrawal Scale (AWS) and refer to AWS guidelines

Step 2. Assess Risk

Consider potential for violence, pulling out tubes, absconding, interference with other patients, falls, accidents etc.

2

If high risk

- Consider a single room
- Companion
- Discuss with senior clinician
- Consider medication

Step 3. Non-Pharmacological Interventions

- Maintain low level sensory stimulation: soft sound & lighting
- Single room if possible
- Avoid multiple room changes and if required complete in daylight hours
- Staff to engage patient calmly, distract and supervise
- Avoid confrontation e.g. walk away, defer interventions or offer food and drink to diffuse tension
- Consider 1:1 companion
- Encourage family to stay and assist

3

Pharmacological Management of Agitation in Older Patients

Antipsychotic medications are not first line treatment and should be used with caution because of the association with strokes and cardiac events. Please reference the High Risk Medicines Policy

Patient with parkinsonism: Use ONLY Quetiapine 12.5mg to 25mg (tablet) 4 hourly PRN. Max 50mg in 24 hours. DO NOT move onto step 4 or 5. If there is no response at this stage seek advice from a senior clinician.

Step 4. Use only ONE of these high potency antipsychotics.

These agents are calming without much sedation
1. Risperidone 0.25 to 0.5 mg (tablet or liquid) 4 hourly PRN Max. 2mg in 24 hours OR
2. Haloperidol 0.25 to 0.5 mg (tablet, liquid, IM) 4 hourly PRN Max. 2mg in 24 hours

4

If the response is inadequate escalate to senior staff
 Upon senior review, proceed to Step 5

Step 5. Use only one of these more sedating antipsychotics

1. Quetiapine 12.5mg - 25mg (tablet) 4 hourly prn - Max 50mg in 24 hours
OR
1. Olanzapine 2.5 mg (tablet) 4 hourly prn - Max 7.5mg in 24 hours

5

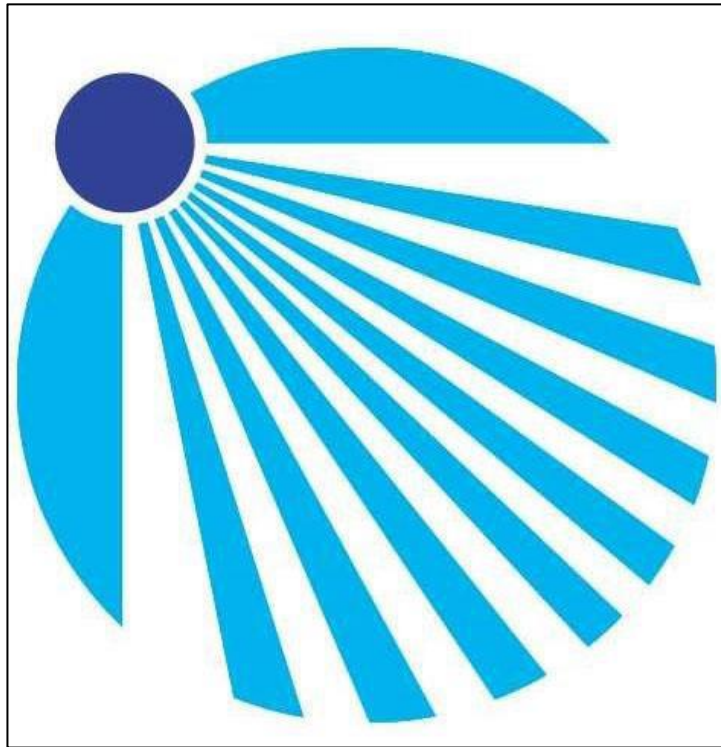
- ✓ Aim to use ONE drug and optimise first line
- ✓ Keep doses to a minimum: giving higher doses on the first night is unlikely to help
- ✓ If the situation is escalating get advice from a senior colleague
- ✓ Review prescription daily
- ✓ Refer to Department of Geriatric, Acute and Rehabilitation Medicine (GARM) or Older Adult Psychiatry during office hours

Step 6. Discharge Planning

Document management plan for de-escalation of antipsychotic or sedating medication in the medical record and on the discharge plan.

6

Appendix 5: Cognitive Impairment Identifier



Appendix 6: Behaviours of Concern (BoC) Escalation Pathway



USE PATIENT LABEL WHEN AVAILABLE

SIR CHARLES GAIRDNER HOSPITAL		URN: 	
BEHAVIOURS OF CONCERN (BOC) ESCALATION PATHWAY		Surname: 	
3 Point Patient ID Confirmed: YES		Forename: 	
Gender: 		DOB: 	
Initiated by: 		Signature: 	
		Date / Time: 	
This form is used when there are Behaviours of Concern, including NEW Confusion or Cognitive change. This pathway provides guidance on further investigations and screening tests and is aimed to provide a framework to apply clinical judgement. Suggested investigations are not mandatory, particularly if done in the last 24hrs - if in doubt, discuss with senior clinician. - Side 1 can be initiated / completed by nursing or medical staff - Side 2 should be completed by medical staff			
4AT Tool Delirium and cognitive impairment	Circle score in each section on the right and total the score		Score
	Alertness Patient markedly drowsy (e.g. difficult to rouse and/or obviously sleepy during assessment) or agitated / hyperactive . Observe patient, if asleep attempt to wake. Ask the patient to state their name and address to assist rating.	Normal (fully alert, but not agitated)	0
		Mild sleepiness for <10 seconds after waking, then normal.	0
		Clearly abnormal	4
	AMT 4 age, date of birth, place (hospital), current year	No mistakes	0
		1 mistake	1
		2 or more mistakes or untestable	2
	Attention Ask the patient: "Please tell me the months of the year in backwards order, starting at December"	Achieves 7 months or more correctly	0
		Starts but < 7 months / refuses to start	1
		Untestable (cannot start because unwell, drowsy or inattentive)	0
Acute change or fluctuating course Evidence of significant change or fluctuation in alertness, cognition, other mental function (e.g. paranoia, hallucination) arising over the last 2 weeks and still evident in the last 24hrs	No	0	
	Yes	4	
If scored 1-3 cognitive impairment is suggested If scored 4 or more, this is possible delirium +/- cognitive impairment		Total Score (4 or more proceed to page 2)	
Behaviours of Concern	Concern about patient actions or behaviour		
	Aggressive, threatening or violent behaviour	No	Yes
	Absconding behaviour	No	Yes
	Conversation illogical and hard to follow	No	Yes
	Hallucinations or delusions	No	Yes
	Concerns about mood	No	Yes
	Self-harm or suicidal behaviour	No	Yes
Others (specify) 	No	Yes	
If ANY concerns about behaviours proceed to page 2 , consider initiating Behavioural Observation Form (BoF) If ANY concerns about behaviours AND there is HIGH SUSPICION that this is driven by mental illness, consider early escalation to senior medical officer and referral to Psychiatry			
Psychiatry / Mental Health Referral ☐			
For all patients on the Behaviours of Concern (BOC) Pathway aim to: Optimise environmental factors Utilise family / familiar people to provide reorientation, reassurance and support Ensure that visual (glasses) and hearing aids are available and utilised Minimise visual and auditory (noise) overstimulation Minimise unnecessary bed moves Avoid the following Confrontation (try to redirect and reorientate) If possible, avoid or delay unnecessary investigations or interventions such as urinary catheterisation Pharmacological management and restrictive practices should ONLY be used WHEN NECESSARY , such as when symptoms are threatening patient or staff safety. Refer to appropriate guidelines on: <u>Management of Agitation in Older Patients with Dementia or Delirium or Management of Acute Severe Behavioural Disturbance</u>			

MR887 BEHAVIOURS OF CONCERN (BOC) ESCALATION PATHWAY

M541
09/25

USE PATIENT LABEL WHEN AVAILABLE

SIR CHARLES GAIRDNER HOSPITAL		URN:					
BEHAVIOURS OF CONCERN (BOC) ESCALATION PATHWAY		Surname:					
3 Point Patient ID Confirmed: YES		Forename:					
		Gender:	DOB:				
Initiated by:		Signature Date / Time					
This page provides a framework to approach the assessment of patients with Behaviours of Concern or NEW Confusion or Cognitive change. It is not intended to replace clinical judgement, not all elements or triggers may be relevant. Underlying mental health disorders may co-exist with organic pathology, and this should be appropriately considered and assessed.							
	Ix	Aetiology for BOC	Potential Investigations (circle as required) Other				
Common Triggers	☐	Urinary retention	Bladder scan				
	☐	Constipation	Stool chart	Refer to MR885			
	☐	Hypoglycemia	Blood glucose	Refer to MR846			
	☐	Pain	Ensure adequate analgesia. Investigate further as required				
	☐	Hypoxia and Hypercapnia	Oxygen saturations. VBG or ABG. CXR				
Acute organic illness	☐	Sepsis	Septic screen – Urinalysis / Urine m/c/s, CXR, Blood cultures	Sepsis Pathway MR 878			
	☐	Metabolic disturbance	Check UECs / Ca / Mg / Phos / TFTs / nutritional screen				
	☐	Acute coronary syndrome	ECG / Troponin	Call Cardiology			
	☐	Acute neurological event	CT head / angiogram	Code STROKE			
	☐	Other acute pathology	Consider other medical comorbidity or surgical illness				
Drug withdrawal	☐	Drug toxicity / adverse reaction	Review new / recent medication changes				
	☐	Alcohol withdrawal	Refer to appropriate withdrawal forms available for alcohol, cannabis, benzodiazepines and methamphetamines	Refer to MR410			
	☐	Nicotine withdrawal					
	☐	Other substances	Refer to Alcohol and Drug services				
Mental Health	Behavioural disturbances should only be attributed to a Mental Health Disorder after organic causes are excluded						
	PHQ-4 Questionnaire for Anxiety and Depression ¹		Not at all	Several days	More than half the days	Nearly every day	Score
	Over the last 2 weeks, how often have you been bothered by the following problems?		(Circle appropriate score)				
	Feeling nervous, anxious or on edge		0	1	2	3	
	Not being able to stop or control worrying		0	1	2	3	
	Feeling down, depressed or hopeless		0	1	2	3	
	Little interest or pleasure in doing things		0	1	2	3	
	Psychological distress, Severe (Score 9-12), Moderate (6-8), Mild (3-5), Normal (0-2)						
	Score of 3 or more for first 2 questions suggests anxiety						
	Score of 3 or more for last 2 questions suggests depression.		Total Score				
Mental State Exam ☐		→	Document in inpatient notes ☐				
Senior Medical Review ☐		→	Further investigations initiated ☐				
Psychiatry / Mental Health Referral ☐		Specify:					
Initial Management / Interim plan:							
Refer to inpatient notes ☐							

1) Kroenke K, Spitzer RL, Williams JB, Löwe B. An ultra-brief screening scale for anxiety and depression: the PHQ-4. Psychosomatics. 2009 Nov-Dec;50(6):613-21